Clinical-information training revision

Terminology and boundaries for routine clinical-information contacts

AUDIENCE	SCOPE	STATUS
Operations & delivery staff; Heronbank Foundry oversight reviewer	England and Wales as at 6 October 2025	Revised training information Prepared by Sofia Adeyemi, Nurse Practitioner

Purpose and bounded scope

This revised clinical-information training document gives operations and delivery colleagues one clear way to describe routine clinical-information contacts. It also enables the Heronbank Foundry reviewer to see the material revision and its boundaries. It is focused training information: it is not patient-specific clinical guidance, a treatment protocol, a triage aid, or a complete contact pathway.

The terminology convention

The earlier draft used “follow-up” for distinct meanings: a scheduled clinical review and any later contact. That ambiguity can make otherwise routine communication difficult to compare. This revision reserves the term for the scheduled clinical-review meaning only.

USE THIS LABEL	MEANING IN THIS REVISION
Scheduled clinical review A planned, identifiable clinical review. “Scheduled follow-up” may be used as its shortened form; do not use the standalone label “follow-up”.	**Later contact** Any later interaction that is not being described as a scheduled clinical review. “Routine later contact” may be used where the contact is routine.

Apply the distinction

- **Scheduled clinical review:** use this where the contact is planned and identifiable as a clinical review. The label describes that planned review relationship, not a frequency, indication or outcome.
- **Later contact:** use this for a later interaction that was not arranged and identified as a scheduled clinical review, including a routine information contact or an unplanned communication.
- **When wording is unclear:** do not infer that a later contact was a scheduled clinical review. Use “later contact” unless it is being described as the planned, identifiable review.

Bounded exception — deterioration disclosed during an otherwise routine contact

Rare but serious. A disclosure of deterioration during an otherwise routine contact is separate from the ordinary terminology distinction. It must not be presented as merely a routine later contact for the sake of simple wording. This document records that communication boundary only: it does not describe the disclosure, diagnose it, predict an outcome, or direct a clinical response. The case sits outside this document’s ordinary guidance.

REVISION RECORD & REUSABLE SUMMARY

What changed — and what this short document does not claim

Material changes from the earlier draft

REVISION FOCUS	EARLIER DRAFT	THIS REVISION / WHY IT MATTERS
Terminology	Used “follow-up” for both a scheduled clinical review and any later contact.	Reserves the label for a scheduled clinical review and names the other category “later contact”. Staff can classify ordinary communications consistently; review can test one visible convention.
Exceptional boundary	Did not state the rare but serious disclosure of deterioration during an otherwise routine contact.	Records it as a separate, bounded exception. The serious information is retained without turning this short training document into clinical guidance or a full pathway.

Limits that remain visible

The revision is deliberately concise for a limited review window. Clear terms and a bounded exception are a useful fallback; they are not an assurance that every contact pathway, clinical question or documentation context is covered. No local policy, external requirement or clinical action is asserted here. The scope is communication and terminology in routine clinical-information contacts only.

STAFF & OVERSIGHT SUMMARY
For staff: call a planned, identifiable clinical review a “scheduled clinical review” (or “scheduled follow-up” only as its shortened form). Call another later interaction a “later contact”. Do not use the standalone label “follow-up”.
For oversight: verify that this convention is used consistently and that deterioration disclosed during an otherwise routine contact remains a distinct, serious boundary rather than ordinary terminology guidance.
Shared limit: this is focused training information for England and Wales as at 6 October 2025, not patient-specific guidance or a complete clinical pathway.

Review use

This document is intended to be read on its own by operations and delivery colleagues and by the Heronbank Foundry reviewer. Its review value is the explicit convention, the visible exceptional boundary and the stated limits—not a promise of comprehensive coverage.